

MODEL TELEMEDICINE PRACTICE GUIDELINES (ILLUSTRATIVE)

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1. Purpose and Scope

1.1 These guidelines set out the minimum standards a registered medical practitioner must follow when providing consultation through audio, video, or text-based telemedicine platforms. 1.2 They apply to first consultations, follow-up consultations, and emergency triage conducted remotely.

2. Practitioner Eligibility

2.1 A practitioner must hold a valid registration with the State Medical Council and complete a mandatory 4-hour telemedicine orientation module before offering remote consultations. 2.2 Registration must be renewed every 5 years, and the orientation module must be repeated once every 3 years to remain eligible.

3. Consultation Categories and Follow-up Windows

3.1 A first consultation must be followed by an in-person or video review within 6 months if a chronic condition is diagnosed. 3.2 A follow-up consultation for the same condition is valid for up to 90 days from the previous consultation, after which a fresh first consultation is required. 3.3 Emergency triage consultations must be escalated to in-person care within 1 hour if red-flag symptoms are reported.

4. Prescription Rules

4.1 List O medicines (basic, low-risk) may be prescribed at any consultation. 4.2 List A medicines (e.g., for chronic conditions such as hypertension) may only be prescribed from the second consultation onward for the same condition. 4.3 List X medicines (controlled substances, including certain opioids and benzodiazepines) shall never be prescribed through a text-only consultation and require a video consultation with the practitioner's digital signature. 4.4 A telemedicine prescription is valid for 7 days for acute conditions and 30 days for chronic conditions, unless otherwise specified.

5. Record-Keeping

5.1 Every practitioner must retain consultation notes, prescriptions, and patient consent records for a minimum of 3 years from the date of consultation. 5.2 Records must be stored on servers located within the country and encrypted at rest.

6. Non-Compliance

6.1 Prescribing a List X medicine in violation of Section 4.3 shall be treated as professional misconduct and referred to the State Medical Council. 6.2 Failure to maintain records under Section 5.1 shall attract a written warning on first instance and suspension of telemedicine privileges for 90 days on a repeat instance within 2 years.